Hospital Visit — Data Collection Checklist

Print this before the visit. This form is for a Nepal hospital project exploring a digital WISN and inpatient Operations tool. Use it to ask about data availability, definitions, sources, and permission. Leave the spaces blank and write responses by hand.

> **Ethics and privacy:** Before copying records, conducting research interviews/surveys, or collecting identifiable information, confirm university ethics requirements and obtain required hospital approval. If approval is not yet in place, use tomorrow's visit only to discuss feasibility and data availability. Do not copy patient records, patient names, hospital identifiers, staff names, or photographs of records. Request aggregate, de-identified totals. Do not enter patient-identifying details in the app.

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1. Visit details

Hospital/facility: _____

District / Province: _____ Date: _____

Student/team members: _____

Hospital contact and role: _____

Visit purpose: [ ] Feasibility discussion [ ] Data request [ ] Workflow observation [ ] Other: _____

University ethics approval/reference (if issued): _____

Hospital permission/contact/reference: _____

Permission status: [ ] Approved for this activity [ ] Pending [ ] Not yet requested

If approval is pending: do not collect/copy research data or administer a formal survey. Ask only whether the data exist, who owns them, and what approvals are required.

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2. Recommended data period — decide this before requesting extracts

Annual WISN staffing analysis (primary recommendation)

Request **one most recent, complete, consistent 12-month period** for a baseline WISN calculation. Use the hospital's established reporting/fiscal year if its records align with it. Write the exact start and end dates below. Do not mix calendar-year totals with fiscal-year staffing data.

Period start: _____ Period end: _____

Reporting year/name used by hospital: _____

For annual workload, request the **total count per activity for the full 12 months**. You do not need 365 daily records to calculate an annual WISN baseline. If available, ask for the same information by month (12 monthly totals); this helps identify missing months and seasonal patterns before summing to the annual total.

- [ ] One complete 12-month activity total is available.
- [ ] Monthly totals are available for all 12 months.
- [ ] Some months are missing; list them: _____
- [ ] Definitions or recording systems changed during the period; describe: _____

If the thesis will study trends: ask whether the same annual totals and definitions are available for **3–5 years**. Five or more consistent years are preferable for trends/forecasting, but a single complete year can support a clearly labelled cross-sectional baseline. Two years can show a simple comparison but do not, by themselves, make a reliable forecast.

Do not substitute a six-month period for an annual baseline without a justified method. If only six months exist, request month-level totals, explain the limitation, and do not simply double the count unless seasonality and the annualisation assumption are defensible.

Operational census and shift-staffing data (separate from annual WISN)

The Operations workflow records a directly observed patient census at **12:00 AM, 7:00 AM, and 7:00 PM**; actual nurses on duty are recorded at **7:00 AM and 7:00 PM only**. A short **2–4 week feasibility/pilot period** can test whether the hospital can record complete handovers, including weekdays and weekends. It is not enough to claim

year-round or seasonal staffing patterns. For year-round variation, use a longer, pre-agreed sampling plan across months or a full year if feasible.

- [ ] One or two days only (software demonstration, not a research trend).
- [ ] 2–4 weeks (feasibility of repeated complete handover recording).
- [ ] Longer period, approved plan: _____
- [ ] Weekdays included [ ] Weekends included [ ] Holidays/seasonal periods considered

Current application limitation: annual WISN inputs are stored as current values, not archived by year. Do not overwrite one year's workload with another and call the result a historical series. Preserve each year's source file and obtain supervisor approval for a year-versioned dataset/extension. Operations rounds are date-stamped, but the app does not currently provide annual WISN trend charts or forecasts.

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3. Permission and data-protection checklist

Before receiving any extract, confirm:

- [ ] Which committee/university review is required for the thesis and staff survey.
- [ ] Written hospital permission and data custodian approval.
- [ ] Whether an additional national ethics review is required for this study.
- [ ] Data fields allowed, date range, departments, and intended use.
- [ ] Data are aggregate/de-identified; no patient names, hospital numbers, addresses, or staff names are needed for the app demonstration.
- [ ] Secure storage location, access permissions, retention period, and destruction plan.
- [ ] Whether results may name the hospital or must be reported anonymously.
- [ ] Consent process for staff interviews/usability feedback.

Approval conditions / follow-up: _____

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4. Questions to ask hospital staff

Ask the appropriate data custodian, nursing administrator/manager, HR/staffing officer, HMIS/statistics officer, and IT/data-protection contact. Do not ask one person to guess figures they do not own.

A. Departments and nursing units

1. What inpatient nursing units currently operate, and what is each unit's official name?
2. Which services are outpatient? Which Emergency beds are admitted/observation beds rather than walk-in visits?
3. Have unit names, bed functions, or reporting boundaries changed in the requested year?
4. Who can confirm that each workload record belongs to the correct unit?

Notes: _____

B. Annual workload volumes and source records

1. Which register/system records the annual volume for each nursing activity?
2. Can the hospital provide totals for the exact 12-month period above? Can it provide monthly totals?
3. Are volumes counts of visits, admissions, procedures, bed-days, or another measure? Define the unit for every measure.
4. Are months or units missing? Were there changes to definitions, forms, software, or reporting practices?
5. Who validates the totals, and can we record the source and extraction date?

Notes/source names: _____

C. Activity and time standards

1. Does the hospital have a current, approved nursing activity/time-standard list?
2. For each standard: what source, version/date, staff cadre, activity definition, and unit of time does it use?
3. Who approved it, and when was it last reviewed?
4. If no standard exists, what local process could validate one (expert panel, time observation, or another approved method)?
5. Which recurring work is direct service, support (for example handover), or additional activity?

Notes: _____

Do not fill missing time standards with guesses. Mark them unavailable and agree with the supervisor how they will be measured or validated.

D. Available working time (AWT)

1. Which work calendar and reference year should be used?
2. What are scheduled working days, public holidays, annual leave, sick leave, training days, and hours per workday for the relevant nursing cadre?
3. Can the facility provide the written policy/calendar and, where approved, actual leave/training summaries?
4. Do values differ by cadre, contract, or work schedule?

Source/period: _____

Notes: _____

E. Current nurse staffing

1. Can staffing be reported by department and cadre for the same reference period?
2. What is the difference between sanctioned posts, filled/in-position staff, vacancies, staff temporarily assigned elsewhere, and actual on-duty staff?
3. Is monthly staffing available, or only a count on a particular date?
4. What definition should the thesis use for “current staff,” and who will approve that definition?

Record separately if available: sanctioned _____ filled/in-post _____ vacant _____

Temporary transfers/secondments or other adjustments: _____

For the WISN input, define the in-post staffing figure and date/period explicitly. The app's `current_staff` is a single annual-planning headcount; it is not an on-duty shift count and does not store cadre breakdowns or yearly history.

F. Operations handovers and local targets

1. Where is the observed patient census recorded at midnight, 7 AM, and 7 PM?
2. Can the hospital identify admitted/observation-bed patients separately from Emergency walk-ins?
3. How is actual on-duty staffing confirmed at the two handovers? Are float/temporary staff counted, and by which unit?
4. Can the hospital record a complete set of units at every sampled round, including weekends?
5. Does a written unit- and shift-specific nurse-patient target exist? Ask for its source, version, effective dates, approving authority, and any minimum staffing rules.
6. Who is authorised to approve or review an aggregate staffing suggestion?

Notes: _____

No default operational ratio is supplied by the application. Do not activate an unapproved value or present a training-only ratio as hospital policy.

G. Existing planning workflow and user needs

1. How are annual staffing requirements calculated today? Who prepares and reviews them?
2. What takes the most time or causes the most errors?
3. What decisions should a dashboard/report help the user make?
4. Which roles may enter data, view results, approve standards, or review suggestions?
5. What concerns would staff have about accuracy, workload, confidentiality, or using a digital tool?

Notes: _____

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5. Department inventory sheet (repeat or attach more copies)

One row per department/unit. Use a short code rather than patient or staff identifiers.

Unit code/name	Inpatient / OPD / Emergency	WISN included?	Operations included?	Data custodian/source
1. _____	_____	_____	_____	_____
2. _____	_____	_____	_____	_____
3. _____	_____	_____	_____	_____
4. _____	_____	_____	_____	_____
5. _____	_____	_____	_____	_____
6. _____	_____	_____	_____	_____
7. _____	_____	_____	_____	_____
8. _____	_____	_____	_____	_____
9. _____	_____	_____	_____	_____
10. _____	_____	_____	_____	_____

Units excluded and reason: _____

6. Annual WISN data collection sheet (repeat for every unit)

Department/unit code: _____ Cadre: _____

Reference period start: _____ end: _____

AWT source and values

AWT item	Value	Source / note
Working days per year	_____	_____
Public holidays	_____	_____
Annual leave days	_____	_____
Sick leave days	_____	_____
Training days	_____	_____
Working hours per day	_____	_____
Computed AWT hours/nurse/year	_____	_____

Staffing snapshot/definition

Current in-post nurses used for WISN: _____ Date/period: _____

Sanctioned: _____ Vacant: _____ Temporary assignments: _____

Staffing source/definition and approver: _____

Workload activity register

Activity name / definition	Type (HS / support / additional)	Time standard (hours)	Annual volume / unit	Source and validation note
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missing data, exclusions, or changes in definition: _____

7. Operations observation sheet (repeat for each date and complete round)

Observation date: _____ Observer code: _____

Round is complete for every inpatient unit: [ ] Yes [ ] No — explain: _____

Unit code	Time	Observed inpatient patients	Nurses actually on duty*	Source / note
_____	12:00 AM	_____	N/A	_____
_____	7:00 AM	_____	_____	_____
_____	7:00 PM	_____	_____	_____

*Record actual on-duty nurses at 7 AM and 7 PM only. Midnight is census-only. Use one line per unit and time; copy this sheet or add lines so all inpatient units are covered. Do not enter patient or nurse names.

Emergency counted only admitted/observation-bed patients: [ ] Yes [ ] No [ ] N/A

OPD excluded from Operations: [ ] Yes Round/data source: _____

Correction/re-entry needed? [ ] No [ ] Yes — reason: _____

8. Optional staff feedback form — only after required ethics approval/consent

This short form is a **draft for feasibility/usability feedback**, not a validated research scale. Ask your supervisor to approve the instrument and sampling plan. Do not record names. Use: 1 = strongly disagree, 2 = disagree, 3 = neither, 4 = agree, 5 = strongly agree, N/A = not applicable.

Participant code: _____ Role category: _____ Date: _____

After a brief demonstration or test task, circle one answer for each statement:

1. I could find the department and activity entry screens. 1 2 3 4 5 N/A
2. The department fields and labels were understandable. 1 2 3 4 5 N/A
3. The difference between annual WISN headcount and on-duty staffing was clear. 1 2 3 4 5 N/A
4. The activity types and annual-volume fields were understandable. 1 2 3 4 5 N/A
5. The dashboard results were easy to interpret. 1 2 3 4 5 N/A
6. I could identify the source/version of an Operations target. 1 2 3 4 5 N/A
7. I understand that Operations suggestions require human review. 1 2 3 4 5 N/A
8. I would need training before using this tool for work. 1 2 3 4 5 N/A

Most useful part: _____

Most confusing/risky part: _____

What data or workflow is missing? _____

Would you use the tool if approved and supported? [ ] Yes [ ] No [ ] Unsure Why?
_____

9. End-of-visit close-out

- [ ] I confirmed the exact annual reference period and data owners.
- [ ] I recorded the source and definition for every requested figure.
- [ ] I distinguished annual WISN staffing from handover on-duty staffing.
- [ ] I confirmed which Emergency counts are admitted/observation beds.
- [ ] I confirmed OPD's WISN/Operations treatment.
- [ ] I have not copied identifiable patient or staff information.
- [ ] I know the next approval step and who must authorize it.
- [ ] I recorded missing data and changes in definitions instead of guessing.

Next person to contact / action / due date:

Researcher initials: _____ Date: _____

Quick recommendation

For your first substantive WISN analysis, request **one complete, recent 12-month period** of aggregate workload data plus the matching AWT and clearly defined staffing figure. Ask for monthly breakdowns if they are available, but do not request 365 daily rows just to total annual workload. Ask for 3–5 historical years only if your approved research question includes trends and the definitions are comparable. Treat Operations as a separate pilot: a few weeks can test feasibility, while year-round claims require a longer, pre-specified observation plan.